



PROCEDURE

Person of Concern

Scope (Staff):	All NMHS Mental Health (MH) Clinical Staff
Scope (Area):	All NMHS MH Inpatient Services

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1. Aim

Patient/consumer with complex needs and risks are identified as being a person of concern. This procedure is to ensure that safe, effective person-centred care is provided and communicated across the care continuum to minimise risk, promote well-being and optimise health outcomes.

2. Background

The [Sustainable Health Review](#) enduring strategy two (2) identifies that mental health care must be integrated with physical health care to improve health and well-being outcomes. The failure to recognise and respond appropriately to acute mental and physical deterioration has been highlighted as a significant factor in a number of adverse events ([WA Your Safety is in our hands, 2020](#)).

This protocol is to be read in conjunction with the following:

- [MP 0181/24 DoH Safety Planning or Mental Health Consumers Policy](#)
- [MP 0181/24 DoH Safety Planning Procedures for Mental Health Consumers](#)
- [MP 0122/19 DoH Clinical Incident Management Policy](#)
- [MP 0155/21 DoH State-wide Standardised Clinical Documentation for Mental Health Services](#)
- [MH Clinical Risk Assessment and Management Policy](#)
- [MH Recognising and Responding to Acute Deterioration Policy](#)

This protocol highlights the importance of identifying persons of concern so that clinical issues are identified, communicated and care is prioritised and strategies are in place to promote and maintain their safety and wellbeing in Mental Health (MH) inpatient and community services.

Services are required to have clinical processes in place to facilitate the transfer of information (such as clinical handover, shift reports, referral documentation and care transfer summaries) and provide evidence of care planning and risk management at transitional points in care in accordance with the requirements of the [DOH Clinical Handover Policy and Guideline](#) and the [Statewide Standardised Clinical Documentation for Mental Health Services](#) (SSCD).

3. Risk

Failure to communicate priority clinical and operational issues in a timely manner may compromise the safety and wellbeing of individuals accessing mental health services.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Principles and Standards

All staff are required to comply with the requirements of the [Mental Health Act 2014](#), the [Chief Psychiatrist's Clinical Standards](#), the [National Standards for Mental Health Services](#) and mandatory policy and reporting requirements.

The [WA Aboriginal Health and Wellbeing Framework 2015–2030](#) identifies the following set of guiding principles and priority areas to improve the health and wellbeing of Aboriginal people in WA:

- Cultural security

-
- The health and wellbeing of Aboriginal people is everyone's business
 - Partnerships
 - Aboriginal community control and engagement
 - Access and equality
 - Accountability

The framework identifies the following strategies for building cultural responsiveness into health services to strengthen Aboriginal health outcomes.

- A culturally competent workforce that has the clinical and cultural training to make them competent practitioners is required. This will enable the health system to respond in a safe, flexible and timely manner to the needs of Aboriginal people and ensure services are better equipped to be responsive.
- Recognising the important role of traditional healing, knowledge and practices will strengthen Aboriginal engagement with the health system. A collaborative and integrated approach to planning and service delivery is required to address duplication and gaps in services.

The [Chief Psychiatrist Clinical Standards 2015](#) identify the following as contributing to positive consumer/patient experience and the delivery of safe and effective care.

All consumers/patients have a right to receive care that is appropriate to their needs and is consistent with their rights under the [Mental Health Act](#) 2014 and the [Charter of Patient Rights](#).

All consumers/patients have a right to receive care that is safe, therapeutic and recognises the following standards:

- Recovery focused, culturally and gender sensitive with an emphasis on promoting recovery through mental and physical healing processes that are individually tailored to the ends of the patient/consumer.
- Balances personal autonomy and decision making with duty of care to provide a safe environment for consumers.
- Trauma Informed by recognising the vulnerability of some consumers due to history, illness, emotional trauma or specific need.
- Recognises the need for routine identification of risk (including sexual risk) for all consumers/patients who receive care from mental health services and the necessity to undertake clinical observations and deliver treatments that are non-intrusive, respectful and sensitive to individual patient needs.
- Promotes therapeutic relationships
- Establishes safety and monitoring activities and promoting a positive and respectful culture, reducing restrictive practices, clinical incidents, adverse events and promoting personal autonomy, self-determination and choice.

5. Recognising Person of Concern

A person of concern is someone that a clinician is particularly concerned about, as defined by the treating clinician. This includes a person admitted to a mental health facility in the previous twenty four hours, a person who had a medical or mental health emergency in the previous twenty four hours, a person being monitored for mental or physical deterioration, a person

discharged from a mental health service requiring community follow up and any other person of clinical concern.

5.1 Persons of Concern may be identified from the following group

- Post MET or Code Blue (for 24 hrs unless clinically indicated to be extended beyond this timeframe)
- Newly admitted within the last 24hrs
- Admitted for Clozapine Initiating Therapy
- Post fall (for 24 hrs unless clinically indicated to be extended beyond this timeframe) as per post falls guidelines ([NMHS Falls Prevention and Management Policy](#))
- With infections or known infectious illnesses, invasive, undergoing chemotherapy
- With wounds, pressure injuries requiring monitoring/interventions
- At risk or identified as at risk of dysphagia
- With respiratory difficulties
- Experiencing dehydration/urinary retention
- Palliative care patients
- Outside normal parameters on ORC – indicating clinical deterioration requiring escalation of care. Consider *could it be Sepsis?*
- Patients experiencing pain or distress not responding to available treatments
- Acute clinical concern relating to health deterioration not already identified above
- Worry or concern from clinicians, patients, families, carers and others

5.2 Persons of Concern may be identified from the following

- With identified risk relating to self-harming behaviour
- With identified risk relating to impulsive behaviours
- Experiencing high levels of emotional arousal/stress/ anxiety and have difficulties with emotional regulation, unclear boundaries
- With identified risk relating to suicide
- With identified risk associated with aggressive or assaultive behaviour
- With neurological disorders, cognitive deficits, sensory deficits
- Medication related problems affecting health (overuse / underuse / complications)
- With identified vulnerabilities associated with Family and Domestic Violence (FDV)
- With sexual vulnerabilities (young people, elderly, U-18 years in adult services)
- With diversity needs, LGBTQIA+
- Aboriginal people
- Experiencing difficulty communicating (CALD)
- Unable to give informed consent
- With Eating Disorders, Body Dysmorphia/Body Image
- Acute clinical concern relating to health deterioration not already identified above
- Worry or concern from clinicians, patients, families, carers

6. Clinical Processes

The Clinical Nurse Specialist (CNS) /Case Manager will be responsible for ensuring that all 'Patients of Concern' have an active alert in (PSOLIS, Alert sticker, iSoft)

Persons of Concern are identified in PSOLIS, on Journey Boards/Handover Reports in community and inpatient settings and flagged during assessment, care planning, risk management and transfer of care.

7. Clinical Handover

Clinical teams will use the standardised clinical handover framework (iSOBAR) as part of a comprehensive, system-wide strategy, that will aid effective, concise and inclusive clinical communication in all clinical situations and contribute to improved patient safety.

Clinical Handover does not guarantee quality care as the information handed over must be acted upon as directed by the [WA DoH Clinical Governance, Safety and Quality Policy Framework](#) and the [DOH Clinical Handover Policy and Guideline](#).

The clinician handing over care /transferring care or referring a person to another service is responsible for ensuring that clinical information is communicated, and that verbal and written information is provided along with risk management plans.

7.1 Handover of Information in Inpatient Settings

Person of Concern will be identified on iSoft (Clinical Handover Module) as a 'Person of Concern' via an alert.

The Shift Coordinator will be responsible for ensure handover of all 'Persons of Concern' to the relevant team at the next shift/working to review and identify a clear management plan. The medical team will determine the level of ongoing care required.

8. Clinical Documentation for Mental Health Services

The clinician handing over care (e.g. leave cover/discharging, transferring care or referring a person to another service) is responsible for ensuring that clinical information is communicated, and that verbal and written documentation is provided along with risk management plans in accordance with the [NMHS Clinical Documentation Policy](#), the [Chief Psychiatrist's Guidelines](#) and the requirements of SSCD.

Individuals, families, carers and other service providers involved in care may receive information (with permission) in verbal and written forms relating to discharge follow-up, emergency contacts and how to re-enter the service if required.

Clinical documentation is undertaken using the identified documentation process in accordance with service requirements and the mandatory tools identified in the SSCD Triage to Discharge Framework (Refer to **Appendix 2**).

9. Discharge/Transfer of Care

Discharge Planning is undertaken in accordance with clinical documentation requirements for the service and the mandatory tools identified in the SSCD Triage to Discharge Framework.

- Clinical Handover (clinician to clinicians discussion and documentation)

- Multiagency communication/accommodation Sector/NDIS/
 - Private Psychiatrist (Consultant to consultant referral)
 - GP/Shared Care
 - Treatment Support and Discharge Plan (TSDP)
 - Discussion of TSDP with consumer/patient, family, carer, support person
 - NaCS - Medical Discharge Summary
 - Care Transfer Summary
 - Risk Assessment and Management Plan (SMHMR905)
 - Coping and Safety Tool for Awareness and Recovery PMR 10
- Patients of Concern (as per the inclusion criteria below) will be identified on ISoft (Handover Module) as a 'Patient of Concern' via an alert.
 - The Shift Coordinator will be responsible for ensuring that all 'Patients of Concern' have an active alert in Isoft by the end of the shift.
 - The Shift Coordinator will be responsible for ensuring that all 'Patients of Concern' are handed over to MDT at the next working day handover meeting.
 - Post-handover, the Shift Coordinator, CNS and Registrar will attend the wards to review the 'Patients of Concern' and amend the care plan and risk assessment accordingly.
 - MDT agreement is required regarding removing patients from the list (patients no longer a cause of concern).

10. Professional Education and Training

Staff are required to comply with the [MHPHDS Mandatory Training Policy](#). This includes completion of annual Basic Life Support Training, PART and knowledge of the Mental Health Act (2014).

All clinical staff must be conversant with:

- PSOLIS and SSCD clinical documentation requirements
- Medication management (doctors and nurses)
- Emergency Management Procedures.

Education on the recognition and response to acute deterioration for medical and nursing staff is included in mandatory annual refresher training.

11. Compliance, monitoring and evaluation

- Review of SAC1 and SAC 2 incidents relating to persons of concern
- Review of patients absconding from inpatient services
- Review Consumers who Do Not Attend (DNA) for Community MHS
- Annual audit of clinical documentation
- Review of relevant HACs
- The MHPHDS Safety, Quality and Performance is responsible for the auditing and reporting to the MHPHDS Executive (LINK to SQP Audit tools)

Related internal policies, procedures and guidelines

WA Department of Health

[MP 0181/24 DoH Safety Planning or Mental Health Consumers Policy](#)
[MP 0181/24 DoH Safety Planning Procedures for Mental Health Consumers](#)
[MP 0122/19 DoH Clinical Incident Management Policy](#)
[MP 0095/18 DoH Clinical Handover Policy](#)
[WA Clinical Handover Guideline](#)
[MP 0155/21 DoH State-wide Standardised Clinical Documentation for Mental Health Services](#)
[WA DoH Agitation and Arousal PRN Medication Chart \(PMR60D\)](#)

NMHS

[NMHS Emergency Management Policy](#)
[NMHS Clinical Incident Management Policy](#)
[NMHS Falls Prevention and Management Policy](#)

MH

[MH Code Blue Medical Emergency Procedure – Inpatient](#)
[MH Medication: Prescribing Administration and Management for Inpatients Policy](#)
[MH Medication: Prescribing Administration and Management for Community Policy](#)
[MHPHDS Mandatory Training Policy](#)
[MH Clinical Risk Assessment and Management Policy](#)
[MH Recognising and Responding to Acute Deterioration Policy](#)
[MH Management of Clinical Alerts Policy](#)

OCP

[The Chief Psychiatrist Clinical Standards](#)
[The Chief Psychiatrist Clinical Guidelines 2015](#)
[The Chief Psychiatrist Sexual Safety Guidelines 2020](#)
[WA Clinical Handover Guideline](#)

References

[ACSQHC Advisory, Recognising and Responding to Acute Deterioration in a person's Mental State 2020 AS 19/01](#)
[ACSQHC Acute Anaphylaxis Clinical Care Standard 2021](#)
[ACSQHC Fact Sheet, Preventing Pressure Injuries and Wound Care](#)
[ACSQHC Evidence for the safety and quality issues associated with the care of patients with cognitive impairment in acute care settings: a rapid review 2013](#)

[ACSQHC Clinical Care Standard for Delirium 2021](#)

[ACSQHC Sepsis Clinical Care Standard 2022](#)

[ACSQHC Cognitive Impairment](#)

[SSCD Statewide Standardised Clinical Documentation: Triage to Discharge V11, 2021](#)

Useful resources (including related forms)

[WA Clinical Alert \(MedAlert\) Policy](#)

[WA Medication Chart Policy MP 0078/18](#)

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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☒  Std 4: Medication Safety		☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management,				
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29/07/2022	V1.1	Director of Nursing MHPHDS	Minor update Glossary of Terms
01/07/2024	V1.2	Policy Officer	Hyperlink update

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD800).

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Appendix 1: Glossary of Terms

Aishwarya's Care Call (ACC): This is an escalation process that is available to patients, carers and family members if they are concerned or worried about the care provided in the event of physiological or mental state deterioration.

Allergy: Occurs when a person's immune system reacts to substances in the environment that are harmless to most people. These substances are known as allergens and are found in dust mites, pets, pollen, insects, ticks, moulds, foods and some medicines.

Anaphylaxis: A severe form of allergic reaction, potentially life-threatening, characterised by a sudden onset in which the clinical presentation is variable. Skin features are not always present.

Aspiration pneumonia: is an infection of the lungs that occurs when the swallowing process fails and a substance (fluid, food, medicines) goes through the trachea to the lungs causing choking or pulmonary aspiration.

Assessment: A clinician's evaluation of a disease or condition based on the person's subjective report of the symptoms and course of the illness or condition, and the clinician's objective findings. These findings include data obtained through laboratory tests, physical examination and medical history; and information reported by family members, carers and other members of the healthcare team.

Basic Life Support: Refers to a nationally accredited or hospital-based training that gives the person skills to undertake Cardio-Pulmonary Resuscitation and maintain life by safe use of defibrillation and the maintenance of a clear airway.

Carer: A person who provides personal care, support and assistance to another individual who needs it because they have a disability, medical condition (including a terminal or chronic illness) or mental illness, or they are frail or aged. An individual is not a carer merely because they are a spouse, de facto partner, parent, child, other relative or guardian of an individual, or live with an individual who requires care. A person is not considered a carer if they are paid, a volunteer for an organisation, or caring as part of a training or education program (ACSQHC Anaphylaxis CCS).

Clinical Care Standards: Statements that include recommendations intended to optimise patient care and are informed by a systematic review of evidence and an assessment of the benefits and harms of alternative care options.

Clinician: A trained health professional, including registered and non-registered practitioners, who provides direct clinical care to a person of concern. Clinicians may provide care within a health service organisation as an employee, a contractor or a credentialed healthcare provider, or under other working arrangements. They include nurses, midwives, medical practitioners, allied health professionals, paramedics, community and hospital pharmacists and other professions who provide health care, and students who provide health care under supervision.

Clinical handover: refers to any situation in which professional responsibility and accountability for some or all aspects of care for a patient, or group of patients, is transferred to another person or professional group on a temporary or permanent basis. The purpose of clinical handover is to achieve effective, high-quality communication of relevant clinical information that is understood and accepted by the receiver when responsibility for patient care is transferred in

accordance with the mandatory DoH policy [WA Health Clinical Handover Policy](#) under the WA DoH [Clinical Governance, Safety and Quality Policy Framework](#).

Clinical handover framework: Refers to the structures and processes identified by the DoH to achieve a consistent and standardised approach to inform clinical handovers undertaken by clinicians within Department of Health services.

iSOBAR: Clinical teams use the standardised clinical handover framework as part of a comprehensive, system-wide strategy to aid effective, concise and inclusive clinical communication in all clinical situations and contribute to improved patient safety. iSOBAR refers to the **6 key steps** in the clinical handover process that specify minimum requirements for safe and effective clinical handover; **I**dentify; **S**ituation; **O**bservations; **A**greed Plan / **A**ssessment and **R**eadback/**R**isk/**R**ecommendation (adapted from the [WA Health Clinical Handover Policy](#)).

Code Blue: A Code Blue medical emergency is called when defined criteria indicative of a life-threatening nature are met that require immediate medical response to maintain life.

Consumer: Different language, terminology and expressions are used across the mental health, psychosocial disability, and related human services sectors. In many service environments 'person', 'client' or 'service user' are used. Some organisations or sectors prefer 'consumer', 'person with lived experience' or 'people who access mental health services'. The NDIS uses the term 'participant'. The Mental Health Coordinating Council acknowledges that community-based services may use different language and encourages services to adapt terminology used according to specific circumstances and context. NMHS MH uses the term '*patient*' to refer to inpatient service users and '*consumer*' to refer to users of community mental health services (Mental Health Coordinating Council 2022).

DATIX CIMS: Datix CIMS is the approved WA health statewide enterprise electronic online Clinical Incident Management System(CIMS) which is used to capture and manage clinical incidents that occur within the WA health system.

Delirium: Delirium is a change in mental status that can be triggered by illness, surgery, injuries or adverse effects of medicines. Some people are more likely at risk of delirium than others. People with delirium experience disturbances in consciousness, attention, thinking and perception. These changes develop over a short period of time (usually hours to a few days). Patients with delirium may be agitated and restless or quiet and withdrawn. Delirium is a serious condition associated with increased mortality which is often unrecognised.

Deterioration in mental state: a negative change in a person's mood or thinking, marked by a change in behaviour, cognitive function, perception or emotional state. Deterioration in a person's mental state can be related to several predisposing or precipitating factors, including mental illness, psychological or existential stress, physiological changes, cognitive impairment (including delirium), intoxication, withdrawal from substances, and responses to social context and environment.

Diversity: the varying social, economic and geographic circumstances of consumers who use, or may use, the services of a health service organisation, as well as their cultural backgrounds, religions, beliefs, practices, languages spoken and sexualities (diversity in sexualities is currently referred to as lesbian, gay, bisexual, transgender and intersex, or LGBTI).

Environment: the physical surroundings where health care is being delivered, including the building, fixtures, fittings and services such as air and water supply. The environment can also



include other **patients/consumers**, visitors and the workforce. In community settings environment refers to clinics, centres, hostels, homes and places where consumers are assessed and reviewed.

Evidence-based practice: care where experience, judgement and expertise are integrated with knowledge about effectiveness gained from a systematic overview of all relevant high-quality research evidence.

Functional decline: functional decline is when people lose their physical, social and/or thinking capacity. Functional decline is commonly associated with illness, injury, disability, mental illness and the ageing process, however it is not inevitable and in many instances treatment and therapy can improve outcomes.

Goals of Patient Care (GoPC): The GoPC facilitates proactive shared decision making between the clinician, patient, person responsible and/or family/carer(s), which is aligned to the patient's preferences, needs, values and wishes establishes and documents the most medically appropriate, agreed goals of patient care that apply in the event of the patient's clinical presentation and / or deterioration.

Goals of Patient Care (GoPC) Form: The GoPC Form (MR00H.1) documents, communicates and directs the agreed goals of patient care that will apply in the event of the patient's clinical deterioration and includes GoPC eForm and other approved DoH versions for use in WA Health services.

HACs: Hospital Acquired Complications reported by DoH as a mandatory service KPI.

ICM -iSOFT: Electronic Inpatient Clinical Management System with reporting/updating functions for clinicians to enter clinical information and flag alerts.

Informed consent: a process of communication between a patient and clinician about options for treatment, care processes or potential outcomes. This communication results in the patient's authorisation or agreement to undergo a specific intervention or participate in planned care. The communication should ensure that the patient has an understanding of the care they will receive, all the available options and the expected outcomes, including success rates and side effects for each option.

iSOBAR: Clinical teams use the standardised clinical handover framework as part of a comprehensive, system-wide strategy to aid effective, concise and inclusive clinical communication in all clinical situations and contribute to improved patient safety. iSOBAR refers to the **6 key steps** in the clinical handover process that specify minimum requirements for safe and effective clinical handover; **I**dentify; **S**ituation; **O**bservations; **A**greed Plan / **A**ssessment and **R**eadback/**R**isk/**R**ecommendation (adapted from the [WA Health Clinical Handover Policy](#)).

Leadership: having a vision of what can be achieved, and then communicating this to others and evolving strategies for realising the vision. Leaders motivate people and can negotiate for resources and other support to achieve goals.

Medical Emergency: A medical emergency is defined as any medical problem that could cause death or permanent injury if not treated quickly. This includes but is not limited to anaphylaxis, acute chest pain, acute onset shortness of breath, collapse, hypoglycaemia, loss of consciousness or seizures.

Medication related problems: any event involving treatment with a medicine that has a negative effect on a patient's health or prevents a positive outcome. Consideration should be given to disease-specific, laboratory test-specific and patient-specific information. Medication related problems include issues with medications such as Underuse; Overuse; Use of inappropriate medications (including therapeutic duplication); Adverse drug reactions, including interactions and Noncompliance (ACSQHC providing care for patients with cognitive impairment or at risk of delirium).

Mental State: The ACSQHC in their scoping study of mental health services (2014) reported that there is no nationally agreed definition of deterioration in a patient's mental state. Through extensive research and consultation, the study identified that deterioration is generally understood to be anything that is a change for the worse in a patient's "mental state" and is broadly understood to refer to a person's intellectual capacity, emotional state, and general mental health based on clinical observation and interviewing. Mental state comprises mood, behaviour, orientation, judgement, memory, problem solving ability and contact with reality.

Multi-disciplinary team (MDT): a team including clinicians from multiple disciplines who work together to deliver comprehensive care that deals with as many of the patient's health and other needs as possible.

Neurological Disorder: is any disorder of the nervous system, structural, biochemical or electrical abnormalities in the brain, spinal cord or other nerves that can result in a range of symptoms. Examples of symptoms include muscle weakness, paralysis, poor coordination, loss of sensation, seizures, confusion, pain and altered Levels of consciousness. The WHO identified health inequalities, social stigma and discrimination as major contributing factors to the disability and impact associated with neurological disorders. Neurological disorders include Acquired Brain Injury, Brain Tumours, Stroke, Huntington's Chorea, Muscular Dystrophy, Dementia and Epilepsy.

Palliative care: an approach that improves the quality of life of patients and their families facing the problems associated with life-threatening illness, through the prevention and relief of suffering by means of early identification and impeccable assessment, and treatment of pain and other problems – physical, psychosocial and spiritual. Palliative care:

- Provides relief from pain and other distressing symptoms
- Affirms life and regards dying as a normal process
- Intends neither to hasten or postpone death
- Integrates the psychological and spiritual aspects of patient care
- Offers a support system to help patients live as actively as possible until death

Person of Concern: Person of Concern: A person that a clinician is particularly concerned about, as defined by the treating clinician. This includes a person admitted to a mental health facility in the previous twenty four hours, a person with identified risks, a person who had a medical or mental health emergency in the previous twenty four hours, a person being monitored for mental or physical deterioration, a person discharged from a mental health service referred to Community MHS and any other person of clinical concern ([WA Health Clinical Handover Policy](#)).

Person/patient/consumer: The NSQHSS uses term ‘person with mental health issues’ to describe people who access mental health care. In contemporary, recovery -oriented mental health services the term ‘patient’ is no longer used. The term also reflects the fact that people accessing health care services may experience mental health issues in the absence of a diagnosed mental illness. Where the terms ‘patient’, ‘consumer’ and ‘mental illness’ are used in quoted resources, including actions in the NSQHS Standards, these terms are retained (Source: NSQHSS)

Patient/Consumer: a person who is receiving care in a health service organisation. In NMHS inpatients are frequently referred to as patients and users of community services are referred to as consumers, however in many instances the terms are used interchangeable and synonymously.

Patient/Consumer goals: clinical and other goals are identified for an episode of care and are determined in the context of a shared decision-making process during collaborative care planning and are documented in the PSOLIS Management Plan.

Person-centred care: an approach to the planning, delivery and evaluation of health care that is founded on mutually beneficial partnerships among clinicians and patients. Person-centred care is respectful of, and responsive to, the preferences, needs and values of patients and consumers. Key dimensions of person-centred care include respect, emotional support, physical comfort, information and communication, continuity and transition, care coordination, involvement of carers and family, and access to care. Also known as patient-centred care or consumer-centred care.

Policy: a set of principles that reflects the organisation’s mission and direction. All procedures and protocols are linked to a policy statement.¹

Procedure: the set of instructions to make policies and protocols operational, which are specific to an organisation.¹

Protocol: an established set of rules used for the completion of tasks or a set of tasks.¹

Psychoactive: psychoactive substances are substances that, when taken in or administered into one’s system, affect mental processes, such as cognition or affect. Psychoactive agents include antipsychotics, benzodiazepines, anxiolytics, hypnotics, anticonvulsants, mood stabilisers, opiates, antidepressants and stimulants.

Recovery: Recovery is understood to be more than reduction in clinical symptoms, and as ‘being able to create and live a meaningful and contributing life in a community of choice with or without the presence of mental health issues.

Recovery-oriented practice: Recovery-oriented mental health practice refers to the application of sets of capabilities that support people to recognise and take responsibility for their own recovery and wellbeing and to define their goals, wishes and aspirations.

Seizure: A seizure is a sign of abnormal brain and occurs when the normal pattern of electrical activity of the brain is disrupted. This can cause changes in sensation, awareness and behaviour, or sometimes convulsions, muscle spasms or loss of consciousness. Seizures vary however most are over in less than 5 minutes. Not all seizures are considered epilepsy.



Sepsis: is a life-threatening condition, with a recognised cluster of signs and symptoms caused by the body's response to infection that can lead to tissue damage, multi organ failure and death if not recognised early and treated promptly

Special: Allocation of a designated staff member to provide additional assessment, observation and monitoring of health-related parameters (such as physical, physiological, behavioural and mental state) as directed by medical staff and documented in the management plan.

SSCD: State-wide Standardised Clinical Documentation used in mental health services as a mandatory framework for documenting clinical activity from Triage to Discharge

Trauma Informed: Recognises the vulnerability of some consumers due to past history, illness, emotional trauma or specific need.

Trauma-informed care: Trauma-informed care is an organisational and practice approach to delivering health and human services directed by a thorough understanding of the neurological, biological, psychological and social effects of trauma and its prevalence in society. It is a strengths-based framework that emphasises physical, psychological, emotional, social, cultural and sexual safety for people who have experienced trauma, their families and carers, and service providers.

Therapeutic Engagement: Clinical interactions that are trauma-informed, sensitive to the diverse needs of consumers/patients and are recovery focused. Staff are required to demonstrate respect, collaboration, support, therapeutic optimism and compassion during all interaction with consumers, patients, family, support persons and carers.

Appendix 2: Screening, Assessment and Treatment Forms

- Patients Progress Notes and Clinical Observation Chart (PMR 29)
- ORC Chart (PMR)
- MET/Code Blue Report (PMR000)
- PRN Agitation and Arousal Chart (PMR60D)
- Sepsis Pathway Clinical Observation (PMR52)
- Dysphagia Screen Tool (DST) (PMR14A)
- Seizure Pathway
- Goals of Patient Care (GoPC) Form (MR00H.1)
- Clozapine Monitoring Protocol
- SSCD - Triage to Discharge Framework
- Abbey Pain Management Scale
- Braden Risk Assessment Tool (pressure injury)